For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Ipracip 20mcg per Actuation HFA Metered Dose Inhaler

Generic name

Ipratropium Pressurised Inhalation BP (20mcg/dose)

Composition

Each actuation delivers
Ipratropium Bromide BP(as monohydrate)
(ex-valve) ....20 mcg
Propellant HFA-134a………………………..q.s
Also contains ethanol Ph.Eur......................15%w/w

Description

A one piece filled metal container fitted with a valve with plastic stem and printed with product identification code, batch number and container number. On visual examination, there should be no sign of physical damage such as dents, bulged containers, bent valve stem, and uneven crimping. Printing details should be legible. Colorless solution is obtained after opening the container

Pharmacodynamics

Ipratropium bromide is a quaternary ammonium compound with anticholinergic (parasympatholytic) properties. It appears to inhibit vagally- mediated reflexes by antagonizing the action of acetylcholine, the transmitter agent released from the vagus nerve. Anticholinergics prevent the increase in intracellular concentration of cyclic guanosine monophosphate (cyclic GMP) caused by the interaction of acetylcholine with the muscarinic receptors on bronchial smooth muscle.

The bronchodilation following inhalation of ipratropium bromide is induced by local drug concentrations, sufficient for anticholinergic efficacy at the bronchial smooth muscle, and not by systemic drug concentrations.

Pharmacokinetics

The therapeutic effect of ipratropium bromide is produced by local action in the airways. Therefore, time courses of bronchodilation and systemic pharmacokinetics do not run in parallel.

Following inhalation, dose portions from 10–30%, depending on the formulation, device and inhalation technique, are generally deposited in the lungs. The major part of the dose is swallowed and passes through the gastrointestinal tract. Due to the negligible gastrointestinal absorption of ipratropium bromide, the bioavailability of the swallowed dose portion is only approximately 2%. This fraction of the dose does not make a relevant contribution to the plasma concentrations of the active ingredient. The portion of the dose deposited in the lungs reaches the circulation rapidly (within minutes).

Limited data on total systemic bioavailability (pulmonary and gastrointestinal portions), based on renal excretion (0–24 hours) of ipratropium bromide, suggests a range of 7–28% when delivery is via a nebulizer or a metered-dose inhaler product. It is assumed that this is also a valid range for inhalation from the powder preparation. This is also a valid range for inhalation from the metered aerosol with HFA 134a propellant because the kinetic results (renal excretion, AUC and Cmax) from the HFA formulation and the conventional CFC formulation are closely comparable.

Kinetic parameters describing the distribution of ipratropium bromide were calculated from plasma concentrations after intravenous administration. A rapid biphasic decline in plasma concentrations is observed. The major portion of approximately 60% of the systemic available dose is eliminated by metabolic degradation, probably in the liver. The main urinary metabolites bind poorly to the muscarinic receptors and have to be regarded as ineffective.

Indications

Indicated as a bronchodilator for maintenance treatment of bronchospasm associated with chronic obstructive pulmonary disease, including chronic bronchitis, emphysema and also in asthma

Dosage and administration

Adults (Including the Elderly)
Usually 1 or 2 puffs, three or four times daily, although some patients may need up to 4 puffs at a time to obtain maximum benefit during early treatment.

Children
6– 12 years
Usually 1 or 2 puffs, three times daily.

Below 6 Years
Usually 1 puff, three times daily.

Contraindications

Hypersensitivity to atropine or its derivatives, or to any component of the product. Hypersensitivity to Soya Lecithin or related food products such as Soya beans or peanuts.

Warning and Precautions

Ipratropium bromide is not indicated for the initial treatment of acute episodes of bronchospasm where rapid response is required. Generally, caution is advocated in the use of anticholinergic agents in patients with glaucoma, prostatic hypertrophy, and bladder-neck obstruction.

Patients should be cautioned to avoid getting the inhaler spray into their eyes. This may result in precipitation or worsening of narrow-angle glaucoma, eye pain or discomfort, temporary blurring of vision, and visual halos or coloured images in association with red eyes from conjunctival congestion and corneal oedema. In such cases, patients should stop treatment with Ipracip 20 mcg per Actuation HFA Metered Dose Inhaler and start treatment with mitotic drops.

Drug Interactions

Ipratropium has been used concomitantly with other drugs, including sympathomimetic bronchodilators, methylxanthines, and steroids, commonly used in the treatment of COPD. With the exception of salbutamol, there are no formal studies fully evaluating the interaction effects of ipratropium and these drugs with respect to effectiveness.

<u>Anticholinergic Agents:</u> Although ipratropium bromide is minimally absorbed into the systemic circulation, there is some potential for an additive interaction with concomitantly used anticholinergic medications. Caution is therefore advised in the co-administration of Ipracip 20 mcg per Actuation HFA Metered Dose Inhaler with other anticholinergic-containing drugs.

Pregnancy and Lactation

Pregnancy

Pregnancy Category B. No adequate or well-controlled studies have been conducted in pregnant women. Because animal reproduction studies are not always predictive of human response, ipratropium bromide should be used during pregnancy only if clearly needed.

Lactation

It is not known whether ipratropium bromide is excreted in human milk. Although lipid-insoluble quaternary bases pass into breast milk, it is unlikely that the active component, ipratropium bromide, would reach the infant to an important extent, especially when taken in aerosol form. However, because many drugs are excreted in human milk, caution should be exercised when ipratropium bromide is administered to a nursing mother.

Undesirable effects

The following side effects have been reported.

Frequencies
Very common
Common
Uncommon Rare
Very rare

Immune system disorders
Urticaria: Uncommon
Anaphylactic reaction: Rare
Angio-oedema of tongue, lips, face: Rare

Nervous system disorders
Headache: Common Dizziness:
Common

Eye disorders
Ocular accommodation disturbances: Uncommon
Angle closure glaucoma: Uncommon
Intraocular pressure increased: Rare Eye
pain: Rare
Mydriasis: Rare

Cardiac Disorders
Tachycardia: Uncommon
Palpitations: Rare
Supraventricular tachycardia: Rare
Atrial fibrillation: Rare

Respiratory, Thoracic and Mediastinal Disorders
Cough: Common

Local irritation: Common

Inhalation induced bronchoconstriction: Common
Laryngospasm: Rare

Gastro-intestinal Disorders
Dryness of mouth: Common
Vomiting: Common
Gastro-intestinal motility disorder: Common
Nausea: Rare

Skin and Subcutaneous Disorders
Skin rash: Uncommon
Pruritus: Uncommon

Renal and Urinary Disorders
Urinary retention: Rare
When using Ipracip 20 mcg per Actuation HFA Metered Dose Inhaler for the first time some patients may notice that the taste is slightly different from that of the CFC-containing formulation. Some patients have described the taste as unpleasant.

- Including giant urticaria
- Ocular complications have been reported when aerolised ipratropium bromide, either alone or in combination with an adrenergic beta2-agonist, has come into contact with the eyes.
- As with other inhalation therapy, inhalation induced bronchoconstriction may occur with an immediate increase in wheezing after dosing. This should be treated straight away with a fast acting inhaled bronchodilator. Ipracip 20 mcg per Actuation HFA Metered Dose Inhaler should be discontinued immediately, the patient assessed and, if necessary, alternative treatment instituted
- e.g. constipation, diarrhoea
- the risk of urinary retention may be increased in patients with pre-existing urinary outflow tract obstruction.

Overdose

No symptoms specific to overdosage have been encountered. In view of the wide therapeutic window and topical administration of ipratropium bromide, no serious anticholinergic symptoms are to be expected. As with other anticholinergics, dry mouth, visual accommodation disturbances and tachycardia would be the expected symptoms and signs of overdose.

Storage

Store below 30°C. Do not freeze.

Shelf life

24 months.

Presentation

1 unit aerosol of 200 metered doses per container

Route of Administration

Oral inhalation

Product Registration holder in Malaysia

CIPLA MALAYSIA SDN BHD
Suite 1101, Amcorp Tower,
Amcorp Trade Centre,
18 Persiaran Barat,
46 050 Petaling Jaya,
Selangor, Malaysia.

Manufactured by

CIPLA LTD
Unit II, Plot No 9 & 10, Indore SEZ, Pharma zone, Phase II, Pithampur District-Dhar, MP 454775, INDIA

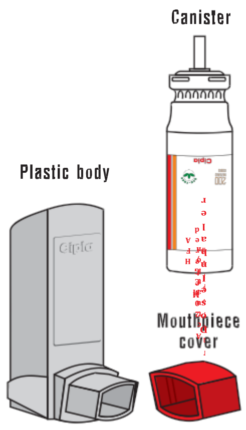
Date of Revision

March 2020

PATIENT INFORMATION LEAFLET

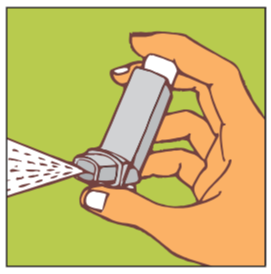
Before using your inhaler read this leaflet carefully and follow the instructions.

Parts of the Inhaler



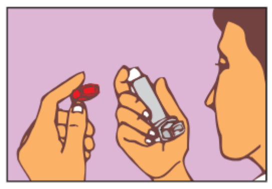
HOW TO USE YOUR INHALER CORRECTLY

Testing your inhaler
Before using your inhaler for the first time, or if it has not been used for a week or more, shake it well and then prime it, i.e. release one puff into the air.



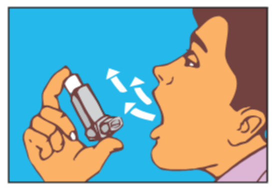
1

Remove the mouthpiece cover, and check that it is clean. Shake the inhaler well. Hold the inhaler upright as shown, with your thumb on the base. Place either one or two fingers on top of the canister.



2

Breathe out gently, through your mouth.



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3

Place the mouthpiece of the inhaler in your mouth between your teeth and close your lips around it (Do not bite it). Tilt your head slightly backwards.

Start breathing in slowly through your mouth. As you breathe in, press down the canister to release one dose while continuing to breathe in steadily and deeply.

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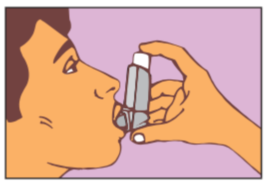
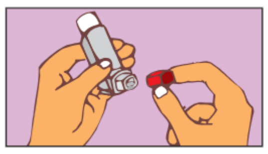


4

Remove the inhaler from your mouth and hold your breath for 10 seconds, or for as long as it is comfortable. Breathe out slowly.

5

If another dose is required, wait for at least one minute and repeat steps 2 to 4. After use, replace the mouthpiece cover.



IMPORTANT:
Do not rush steps 3 and 4. It is important that you start to breathe in as slowly as possible just before releasing the dose. Practice in front of the mirror for the first few times. If you see "mist" coming from the top of the inhaler or the sides of your mouth, start again from step 1. This escaping mist indicates incorrect technique.

FOR CHILDREN

Parents must assist those children who need help in using the inhaler correctly.

CLEANSING

It is essential to keep the plastic mouthpiece clean, to ensure proper functioning of the inhaler. Clean your inhaler at least once a week, as follows:

1. Gently pull the metal canister out of the plastic body of the inhaler. Remove the mouthpiece cover.
2. Immerse the plastic body and the mouthpiece cover in warm water. **Do not put the metal canister in the water.**
3. Next, rinse the plastic body and mouthpiece cover under running tap water.
4. Shake well to remove excess water.
5. Leave to dry in warm place. Avoid excess heat. When the plastic body is dry, replace the canister and the mouthpiece cover correctly.

WARNING

If the recommended dose does not provide the desired effect, consult your doctor. It is dangerous to exceed the recommended dose.

The metal canister is pressurised. Do not puncture or burn it even when empty.

Keep away from eyes.
Keep away from children.

STORAGE

Store below 30°C.
Do not freeze.

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